

# Anxiety and Panic Attack - Female (Child)

Algorithm 1064 | Child (10-17 years) | Gender at birth: Female

## Definition

- Panic attack/anxiety assessment decomposed from NHS.UK's published panic disorder guidance.
- Child patient.
- Female.

## Clinical background

- Panic attacks can cause sudden intense fear with physical symptoms, but emergency cardiac or breathing presentations must be excluded first.
- Repeated unexpected panic attacks followed by persistent worry may indicate panic disorder.
- A single settling episode without emergency features still requires safety-net advice.

## Initial assessment

1. [OPEN\_TEXT] What is the person's exact age and sex, are they pregnant/recently postpartum, when did this episode start, and is it first, different, or more severe than prior clinician-diagnosed panic episodes?
2. [YES\_NO] Has this happened before?
3. [OPEN\_TEXT] Describe what the caller is feeling right now.
4. [CAREGIVER\_OBSERVATION] What change has the parent or caregiver observed in behaviour, alertness, eating or drinking, sleep, school or play, and normal activity?

## Triage and redirect questions

### **1. Redirect → Depression**

Is persistent low mood or loss of interest the main concern, without a current panic episode?

Persistent depressive symptoms belong in the depression pathway; suicide risk must still be screened immediately.

### **1. Disposition level 100**

Is there crushing or pressure-like chest pain, pain spreading to the arm/jaw, fainting, or severe difficulty breathing that does not fit a typical panic attack pattern?

Universal emergency rule-out added ahead of the panic disorder guidance itself (NHS.UK's page notes it does not give 999 criteria) - panic attack symptoms can closely mimic cardiac or respiratory emergencies, which must be ruled out first, not assumed to be anxiety.

### **1. Disposition level 50**

Has the caller had regular, unexpected panic attacks followed by at least a month of ongoing worry about having more?

NHS.UK guidance: this pattern may indicate panic disorder and warrants a primary-care review for assessment.

### **1. Disposition level 15**

Is this a single, isolated panic episode now settling, with none of the features above?

NHS.UK guidance provides in-the-moment coping techniques for an isolated panic episode.

## Advice

### ***Qatar emergency response for symptoms not safely attributable to panic***

- Call Qatar emergency services on 999 and do not drive for first/severe chest pain, abnormal breathing, fainting, stroke signs, seizure, anaphylaxis, pregnancy/postpartum danger signs, poisoning, or immediate risk of harm. Keep the person with you, allow the position that makes breathing easiest, and do not assume a prior panic diagnosis explains new or different symptoms. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: worsening chest pain or breathing; fainting, seizure, or neurological change; risk of self-harm or harm to others.

### ***Routine panic disorder follow-up***

- Book a routine primary-care review to discuss recurring panic attacks and treatment options.
- Call Back If: attacks become more frequent or severe.

### ***Coping with a panic attack***

- Stay where you are if possible, breathe slowly and deeply, remind yourself the attack will pass and is not life-threatening, and focus on calm, positive images.
- Call Back If: symptoms do not settle; chest pain or breathing difficulty develops.

## First aid

- Call Qatar emergency services on 999 and do not drive for first/severe chest pain, abnormal breathing, fainting, stroke signs, seizure, anaphylaxis, pregnancy/postpartum danger signs, poisoning, or immediate risk of harm. Keep the person with you, allow the position that makes breathing easiest, and do not assume a prior panic diagnosis explains new or different symptoms. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

## References

- NHS.UK, "Panic disorder", <https://www.nhs.uk/mental-health/conditions/panic-disorder/> (page last reviewed 22 August 2023)

## Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from the cited NHS.UK pages under the Open Government Licence v3.0; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect and is not a disposition row. Local disposition mappings and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

Manifest hash: ea853daa5fd2162333f2e419438d2bc0eb9d3f4b6f444e70b1efb82e25d4c12f | Status: BLOCKED\_BY\_RESEARCH\_GAP